

158MO **Niraparib with abiraterone acetate and prednisone (NIRA+AAP) as first-line therapy in patients (pts) with metastatic castration-resistant prostate cancer (mCRPC) with homologous recombination repair (HRR+) gene alterations: Asian subgroup analysis of the MAGNITUDE study**

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Background: In the phase 3 MAGNITUDE study (NCT03748641), NIRA+AAP met the primary endpoint of radiographic progression-free survival (rPFS) and improved other clinically relevant outcomes in pts with HRR+ mCRPC. Here we report data from an Asian subgroup analysis of this study.

Methods: A subgroup analysis was conducted for the primary endpoint (rPFS by blinded independent central review [BICR]), secondary endpoints (time to cytotoxic chemotherapy [TTCC], time to symptomatic progression [TTSP], and overall survival [OS]), as well as time to prostate-specific antigen progression (TTPP), overall response rate (ORR), and safety in Asian pts with HRR+ mCRPC. Pts were enrolled at study sites in China, Korea, Malaysia, and Taiwan, and received NIRA 200 mg + AAP or placebo (PBO) + AAP daily.

Results: Sixty-eight Asian pts with HRR+ mCRPC (28 NIRA+AAP; 40 PBO+AAP) were assessed. Median age was 71 yr, 15% had prior AAP, 34% had visceral metastases, and 44% had BRCA2 mutations; no BRCA1 mutations were reported. Median follow-up was 17.7 mo. Improvement in rPFS by BICR was observed with NIRA+AAP vs PBO+AAP, with reduced risk of progression or death of 55% (median 22.0 vs 11.1 mo) (Table). NIRA+AAP delayed TTCC, TTSP and TTPP, and improved ORR. All P-values are nominal. OS data are immature. Grade 3/4 treatment-emergent adverse events (TEAEs) occurred in 68% of pts in the NIRA+AAP group (most commonly anemia [43%]) and 38% in the PBO+AAP group (most commonly alanine aminotransferase increased [13%]). Rates of TEAEs leading to discontinuation of at least one study drug were 11% with NIRA+AAP and 8% with PBO+AAP.

Table: 158MO		
Asian pts with HRR+ mCRPC	HR (95% CI); Nominal P-value	
	BICR	Investigator assessed
rPFS	0.45 (0.20-1.02); P = 0.0514	0.50 (0.20-1.24); P = 0.1273
TTCC	0.00 ^a (0.00-NE); P = 0.0071	
TTSP	0.42 (0.11-1.58); P = 0.1858	
OS	0.62 (0.20-1.88); P = 0.3939	
TTPP	0.36 (0.13-0.97); P = 0.0364	
ORR	Relative risk, 2.75 (0.99-7.62); P = 0.022	

^a10 events were reported with PBO+AAP and 0 events were reported with NIRA+AAP, resulting in an HR = 0.

Conclusions: Data from this Asian subgroup analysis of the MAGNITUDE study are aligned with the global results and support the benefit of NIRA+AAP in HRR+ mCRPC.

Clinical trial identification: NCT03748641.

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159MO **Low-dose abiraterone with fatty food versus standard dose abiraterone in metastatic castration-resistant prostate cancer**

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Background: Standard dose of Abiraterone Acetate (AA) for metastatic CRPC is 1000mg/day in two divided doses to be taken on empty stomach. Food intake has shown alteration in pharmacokinetics of AA. Phase I studies have shown a 4.4 fold increase in drug absorption when taken with a high fat meal. Based on this hypothesis, studies have shown that low dose AA with a fatty meal is non inferior to standard dose. We did a prospective study of AA to confirm the same.

Methods: The primary objective of this study was to compare PSA response rate at 12 weeks with low dose (250mg) versus Standard dose (1000mg) Abiraterone Acetate. It was conducted from Dec 2019 to Dec 2020 and was a prospective randomised open-label study, where 70 consecutive patients with mCRPC were randomized in 1:1 ratio to receive low-dose AA (250 mg with a fatty meal) or Standard-dose AA (1000 mg in fasting state). Both groups received Prednisone at 5 mg twice daily. Recommended options for fatty meal included : a. Two teaspoons of Ghee b. Two teaspoons of Groundnut chutney c. Two teaspoons of clarified Butter.

Results: In the standard dose AA arm 32 out of 35 patients had bone metastasis while 3 patients had both bone and lymph nodal metastases. In low dose AA arm, 23 patients out of 35 had bone metastasis only while 12 patients had both bone and lymph nodal metastasis. Median baseline PSA level in the standard dose arm was 47 and in the low dose arm it was 65. At 12 weeks, in the standard AA arm, 2 patients out of 35 had PSA progression. The mean response to standard dose Abiraterone acetate was 68%. In the low dose AA arm, none of the patients had PSA progression. The mean response rate in the low dose arm was 82%. Fatigue (13 vs 12 patients in standard vs low dose arm respectively), Hypertension (12 vs 10 patients in standard vs low dose) hypokalaemia (11 patients in each arm) and fluid retention (9 vs 3 in standard vs low dose arm) were observed.

Conclusions: In metastatic CRPC, low dose AA taken with fatty meal is similar to standard dose AA in terms of PSA response and tolerance. In low resource countries like India, decreasing treatment costs will increase access to treatment, improve compliance and prevent interruption in treatment.

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